

PRIORITIES:

- ABCs
- Remove from offending environment;
- Assure an advanced life support response
- Determine degree of physiologic distress, identify nature of illness or injury
- If patient is in extremis, begin therapeutic modalities prior to conducting secondary evaluation

Moderate Hypothermia

Body temperature 84 – 95 F (28 – 35 C), conscious and shivering but lethargic, skin pale and cold.

1. Ensure a patent airway
2. Be prepared to support ventilation with appropriate airway adjuncts
3. OXYGEN THERAPY – Begin oxygen at 6 liters/ minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respirations as needed. **DO NOT** withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD.
4. Gently move to sheltered area minimizing physical exertion or movement of the patient;
5. Cut away wet clothing and cover patient with warm, dry sheets or blankets;
6. Monitor vital signs frequently;.
7. Assist advanced life support personnel with patient packaging and movement to ambulance.

Severe Hypothermia

Body temperature 84 F (28 C), stuporous or comatose, dilated pupils, hypotensive to pulseless, slowed to absent respirations.

1. Handle gently, but ensure a patent airway;
2. OXYGEN THERAPY – Begin oxygen at 6 liters/ minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respirations as needed. **DO NOT** withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD.
3. Remove all wet clothing and wrap in dry blankets. **DO NOT** apply an external heat source;
4. Move gently. Rough handling may precipitate cardiac arrest;
5. Assist advanced life support personnel with patient packaging and movement to ambulance;
6. NOTE: Severe hypothermia patients may appear dead. When in doubt, do CPR.

Environmental
E-02 Cold Injury / Hypothermia BLS

Revised 3/1/2000

Frostbite

Areas of skin that are white, numb or burning, soft to touch but do not re-color with touch.

1. Evaluate and treat HYPOTHERMIA;
2. Move to warm environment and wrap affected extremity with thick, warmed blankets or clothing. DO NOT rub or otherwise attempt active re-warming;
3. Monitor vital signs frequently;
4. Assist Advanced Life Support personnel with patient packaging and movement to ambulance;
5. Consider:
 - Protect injured areas from pressure, trauma, and friction;
 - Do not rub or break blisters;
 - Place gauze between toes and fingers;
 - Gradually warm extremities;
 - Do not allow to refreeze;
 - Restrict movement of extremities. Do not allow patient to walk if feet involved;
 - Keep patient warm but not overheated.

Environmental E-02 Cold Injury / Hypothermia

PRIORITIES ➤ ABCs ➤ Maintain airway, provide oxygen and ventilatory support PRN ➤ Remove from offending environment ➤ FROSTBITE TREATMENT: Wrap affected extremity with thick, warm blankets or clothing. Do NOT rub or otherwise attempt active rewarming. ➤ Early Transport; ➤ EARLY CONTACT OF RECEIVING HOSPITAL.	Reviewed JULY 2006
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PATIENT TREATMENT GUIDELINES

- Stabilize airway; handle gently.
- Move patient to a sheltered area, minimizing physical exertion or movement of the patient;
- Cut away wet clothing and cover patient with warm, dry sheets or blankets;
- **OXYGEN** therapy – Be prepared to support ventilations using appropriate airway adjuncts. If spontaneous respirations are present, intubate only if absolutely necessary to prevent aspiration or if ventilations are inadequate;
- **CARDIAC MONITOR.** Observe for organized rhythm and pulses for one minute. If organized rhythm present, move quickly but gently to warm environment. If in asystole or ventricular fibrillation, begin chest compressions and treat per specific cardiac protocols;
- **IV** Access and 250 cc fluid challenge if indicated; repeat vital signs; if BP <100 systolic, repeat Bolus.
- **CONTACT BASE PHYSICIAN FOR ADDITIONAL ORDERS.**

DISRUPTED COMMUNICATIONS

In the event that a Solano County EMT-P is UNABLE to make physician contact for orders, the paramedic MAY NOT utilize those areas of the protocol needing physician direction and MUST TRANSPORT IMMEDIATELY TO THE CLOSEST FACILITY.